

Why Your Census Number Is Not Arbitrary

What Nobody Tells Hospice Sales Reps About the Business They Sell For

A Spartan Coaching Field Education Document | Branch Profitability Series

The Number You Were Given Without an Explanation

At some point early in your hospice sales career, someone gave you a number. Eight admissions a month. Twelve. An ADC of 60 by Q3. Maybe it came with a chart. Maybe it came with a warning. But what it almost certainly did not come with was an explanation of where it came from, what it costs the organization every single day it goes unmet, or — most importantly — what it actually means for the people your branch exists to serve.

That is not a small gap in your education. It is a fundamental one. Because without understanding the why behind the number, you are executing a task without understanding its purpose. And in hospice — where the work you do connects directly to whether a dying person receives expert, compassionate care in their final weeks of life — purpose is not optional context. It is the job.

This document exists to give you what your sales leader did not have time to explain, what your clinical team assumed you already understood, and what your executive director may not have known how to translate into language a rep in the field could actually use.

Read it once. Then read it again when the week is hard and the number feels abstract. Because the number is never abstract. Behind every point of ADC is a nurse making a 2 a.m. phone call, a social worker sitting in a family's kitchen, a chaplain holding someone's hand at the end of a long life. The census is how that work stays funded.

BEFORE THE NUMBERS

What a Hospice Branch Is Actually Doing

Most people outside of healthcare think of hospice as a place you go to die. Most people inside healthcare know it is the opposite. Hospice is where you go to *live* — with comfort, with dignity, with a coordinated team of clinical professionals who take over the medical burden so the patient and family do not have to carry it alone.

When a patient enrolls in hospice, they are placing an enormous amount of trust in your branch. Not just trust in the idea of hospice, but trust in specific people. The RN case manager who will come to their home twice a week and know their name, their pain level, their fears, and their wishes. The hospice aide who will help them bathe and stay clean and feel human. The social worker who will navigate the family dynamics and help a daughter figure out how to stop being a medical coordinator and just be her father's daughter again. The chaplain who will show up without an agenda and sit with whatever the patient needs to say.

"A patient does not just enroll in a benefit. They entrust a team of people with the most intimate chapter of their life — and their death."

That team does not appear because of goodwill. It does not exist on hope or mission statements. It exists because your branch generates enough revenue from enough patients on census to employ, train, schedule, supervise, equip, and retain every one of those people. Every day.

That is the real meaning of the census number. Not a line on a dashboard. Not a quota to satisfy. It is the financial floor below which that care team cannot be sustained — and below which the patients who need them do not receive them.

PART ONE

The People Behind the Cost Structure

Every line item in a hospice budget is a person. Understanding the cost structure of your branch means understanding the people your census supports — and what happens to them, and to your patients, when that census falls short.

The Clinical Team — The Heart of the Branch

These are the people who provide direct patient care. They are not optional staff. Medicare Conditions of Participation require that every hospice maintain these roles regardless of census. They must be hired, trained, credentialed, and retained — and their salaries are paid every two weeks whether the census is 20 or 80.

Clinical Role	Purpose in Patient Care	Annual Cost (Approx.)
Executive Director	Oversees clinical operations, compliance, and team performance	\$140,000
Supervisor RN Case Manager	Manages clinical staff, ensures care quality standards	\$110,000
RN Case Manager (x2 min)	Primary clinical contact; visits patients 2x/week or more	\$100,000 each
Hospice Aide (x2 min)	Personal care: bathing, hygiene, comfort, and human dignity	\$50,000 each
Social Worker	Family support, care planning, grief counseling, discharge navigation	\$75,000
Chaplain	Spiritual support, meaning-making, presence at end of life	\$70,000
After Hours RN	Responds to patient crises overnight — the 2 a.m. phone call	\$95,000
Weekend RN	Ensures continuity of care when the week ends for everyone else	\$95,000
Medical Director (Contract)	Certifies eligibility, oversees pain and symptom protocols	\$75,000

As census grows, so does the clinical team — by Medicare requirement, not by choice. An RN Case Manager is typically added for every 12 patients on service. A Hospice Aide for every 8. The branch cannot delay those hires without risking compliance and, more importantly, without failing the patients already enrolled.

What this means in practice: when census drops, these positions are the last to be eliminated — because eliminating them means abandoning enrolled patients. The branch absorbs the loss. It operates in the red. It draws on reserves. And the pressure falls on the sales team to restore the census that makes those positions financially viable again.

The Operational Team — The Infrastructure That Makes Care Possible

Behind every patient visit is a scheduling system, a documented care plan, a supply order, a referral processed, and an admission coordinated. These roles exist so the clinical team can focus on patients instead of paperwork.

Operational Role	What They Make Possible	Annual Cost (Approx.)
Intake Coordinator	Receives referrals, processes admissions, coordinates first visit	\$60,000
Secretary / Administrator	Scheduling, documentation, billing support, office operations	\$55,000

The Sales Rep — The Person Who Makes All of It Possible

Every other role on this list depends on what you do in the field. The RN Case Manager only has a patient to care for because a referral was made. The social worker only has a family to support because someone had the right conversation with the right physician at the right time. The chaplain only has a purpose in the branch budget because census is high enough to support their position.

Sales Role	What They Generate	Annual Cost (Approx.)
Sales Rep / Market Development	Admissions, referral relationships, census growth	\$115,000

Your salary is a fixed cost paid whether you produce or not. But more than that — every role on this list is funded by the census you build. You are not chasing a quota. You are funding a care team.

PART TWO

What It Costs to Care for Each Patient, Every Day

Beyond salaries, every patient enrolled in hospice generates a daily cost. These are the direct costs of delivering care to one person, one day at a time. They are variable — they scale with census — and they must be paid before a dollar of profit is possible.

Daily Patient Cost	What It Covers	Cost Per Patient Per Day
Pharmacy / Medications	Medication management, comfort medications, symptom control drugs	\$22 - \$44
Durable Medical Equipment	Hospital beds, wheelchairs, oxygen equipment, walkers	\$10
Medical Supplies	Wound care, nursing supplies, personal protective equipment	\$10 - \$23
Clinical Time	Reimbursement for home visits by RNs, aides, social workers	\$6
Other Patient Costs	Volunteer coordination, bereavement mailings, miscellaneous	\$5
Total Daily Variable Cost	Per patient, per day — before a dollar of profit	\$53 - \$88

High-acuity patients — those with advanced heart failure, ALS, end-stage COPD, or complex pain management needs — can push daily medication costs alone past \$80. These patients often have longer lengths of stay, which benefits the branch financially, but their daily care cost is significantly higher and must be factored into any honest financial model.

Monthly Overhead — The Cost of Having a Place to Do This Work

In addition to salaries and direct patient costs, every branch carries a layer of fixed operational overhead that exists before anyone walks through the door. This includes office rent, utilities, liability insurance, professional licensing fees, billing and EMR software, compliance training programs, office equipment, telephone systems, and a range of other administrative costs that have nothing to do with any specific patient but everything to do with being able to operate at all.

Overhead Category	Monthly Cost (Approx.)	Annual Total
Office Rent, Utilities, Insurance	\$18,000 - \$22,000	\$216,000 - \$264,000
EMR / Billing / Compliance Software	\$6,000 - \$8,000	\$72,000 - \$96,000
Training, Licensing, Credentialing	\$4,000 - \$6,000	\$48,000 - \$72,000
Administrative & Miscellaneous	\$8,000 - \$12,000	\$96,000 - \$144,000

Total Monthly Overhead	\$35,000 - \$45,000	\$420,000 - \$540,000 / year
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Before a single salary is paid. Before a single patient medication is purchased. Before a nurse makes a single visit. The branch is spending between \$420,000 and \$540,000 per year just to exist as an operating entity. Census is what covers that cost — and everything above it.

Where the Revenue Comes From

Medicare pays hospice under a per diem rate model called Routine Home Care (RHC). For each day a patient is enrolled in hospice and receiving care, the branch receives a fixed daily payment from Medicare. This rate does not change based on how many visits were made that day or how much medication was administered. It is a flat daily rate — which is why census is the primary driver of revenue. More patients on service means more days of reimbursement.

Medicare Payment Period	Daily RHC Rate (Approx.)	Why It Changes
Days 1-60 of a spell	\$208 - \$224 per patient day	Higher rate reflects higher early-stay clinical intensity
Days 61 and beyond	\$177 - \$209 per patient day	Rate adjusts downward after the initial care period

After subtracting the daily patient costs (\$53-\$88), the remaining amount is called the **contribution margin** — the portion of each patient day that goes toward covering salaries, overhead, and ultimately, profit. That contribution margin is approximately **\$120 to \$155 per patient, per day**.

Every patient on census is generating roughly \$120 to \$155 per day toward the fixed costs that keep your clinical team employed and your branch operational. Every vacant day — a patient who was not admitted, who left service early, who was never referred — is \$120 to \$155 that did not arrive.

PART THREE

The Numbers That Should Have Been Explained to You on Day One

Break-Even ADC — The Floor Below Which Everything Breaks Down

Break-even ADC is the Average Daily Census at which total revenue exactly equals total costs — salaries, daily patient costs, and overhead combined. It is not a goal. It is not a benchmark. It is the minimum viable condition for the branch to function without losing money every single day.

A typical hospice branch requires between 35 and 55 patients on census at all times just to reach break-even. Below that threshold, the branch absorbs a daily operating loss. Those losses drain cash reserves, constrain clinical hiring, and create financial pressure that eventually affects the quality of care — even if no one says so out loud.

What happens when a branch operates below break-even for an extended period? Leadership delays replacing a departing RN. The after-hours coverage gets stretched thin. Supply orders get scrutinized. Training budgets get cut. The clinical team feels the pressure even if they cannot name its source. And the patients — the patients who enrolled because they trusted this organization — receive care from a team that is operating under financial strain.

That is the human cost of a census that stays below break-even. It does not show up on a balance sheet. But it shows up in the field.

Target Margin ADC — The Number That Actually Funds the Mission

Break-even means the branch survived. Target Margin ADC means the branch is sustainable, investable, and able to grow. Healthy hospice organizations operate at an operating margin of 12 to 18 percent. That margin is not profit extracted from patients. It is the financial cushion that allows the organization to do the things that make care better:

- Hire additional clinical staff before census demands require it, so new patients receive full attention from day one
- Invest in training and continuing education that keeps the clinical team sharp and current
- Build the administrative infrastructure that reduces errors and delays in care coordination
- Expand to underserved markets where eligible patients are currently not receiving care
- Weather market disruptions, regulatory changes, and reimbursement adjustments without cutting clinical staff

The difference between break-even ADC and target margin ADC is typically 15 to 30 additional patients on census. Those patients are not a rounding error. They are the difference between an organization that survives and one that thrives — and between a clinical team that is stretched and one that is supported.

Every admission above break-even is not just revenue. It is investment capital for a better clinical operation, a stronger care team, and more patients reached in the future.

The Admissions Math — What You Actually Need to Produce

Census does not hold itself. Patients die. Some discharge. Some revoke. A branch must continuously admit new patients to replace those leaving service. The formula that determines exactly how many admissions are required is driven by two variables: your target ADC and your average length of stay.

Monthly Admissions Needed = (Target ADC x 365) / Average Length of Stay / 12

Target ADC	Avg. Length of Stay	Monthly Admissions Needed	Weekly Admissions Needed
30 patients	60 days	~15 admissions/month	~4 per week
50 patients	90 days	~20 admissions/month	~5 per week
60 patients	90 days	~24 admissions/month	~6 per week
80 patients	90 days	~32 admissions/month	~8 per week
100 patients	90 days	~41 admissions/month	~10 per week

If your branch targets 60 ADC with a 90-day average length of stay, it needs approximately 24 new admissions per month — 6 per week — just to hold census flat. Not grow it. Hold it. If two marketers split that production, each rep needs to close 12 admissions per month. That is not a soft goal. That is the math behind maintaining the care team that is already employed.

PART FOUR**Reading the Field Differently Now That You Know This**

The conversations you have in the field look different when you understand what is at stake behind the census number. Here is how to reread situations you have already experienced.

When your manager says 'we need three more admissions this month,'

They may not be talking about a quota. They may be talking about the break-even line. Three admissions at a 90-day average length of stay adds 7 to 8 patients to the running ADC over the following quarter. At \$135 per patient per day in contribution margin, that is roughly \$340,000 in annual revenue that either arrives or does not. The urgency is real. Now you know why.

When a referral stalls and you do not follow up,

The cost is not a missed commission. It is a patient who did not get care, and a family that navigated end of life without a team behind them. It is also a gap in census that the branch absorbs financially — paying every salary on that list — while the revenue that would have covered it never arrived. Referrals do not stall themselves. They stall in the gap between your last contact and your next one.

When a patient is admitted with a very short prognosis,

A two-week patient barely covers their own admission cost after all direct expenses are calculated. The patients who sustain a branch financially — and who receive the most meaningful hospice care — are those who were referred early, when hospice had months to provide comfort, education, and support. Early referral is not just a financial win. It is the difference between a patient who had time to get to know their care team and one who met them for the first time days before dying.

When the clinical team seems stressed and understaffed,

There is likely a direct connection to census. Below break-even, the branch may be holding open positions, stretching existing staff thin, or delaying hires that the caseload genuinely requires. The RN who has one too many patients on her list is a symptom of a census that is not sustaining the staffing model it needs. Your production is not separate from clinical quality. It is upstream of it.

When you understand that every conversation you have connects to the census,

The work becomes something different. Not a quota to survive. Not a metric to manage. A direct line between your effort in the field and the quality of care that patients receive in their homes. You are not moving referrals. You are funding the team that shows up at 2 a.m. You are sustaining the chaplain who sits with someone who has no family left. You are making it possible for a social worker to tell a family that they do not have to do this alone.

THE BRANCH PROFITABILITY SIMULATOR**A Tool Built to Make This Visible**

The Spartan Coaching Branch Profitability Simulator was built because this education almost never happens in the field. Sales reps are given goals without context. Leaders are given dashboards without explanations. And the connection between daily census, clinical staffing, patient costs, and care quality remains invisible to the people whose work most directly determines it.

What the simulator allows you to model:**Break-Even ADC**

Enter your branch's revenue rates, clinical costs, and staffing assumptions to calculate the minimum census that covers all expenses

Target Margin ADC

Set your operating margin goal and see exactly what census is required to achieve it — your real sales target

Full Staffing Cost Model

See how clinical and operational payroll scales as census grows, including which positions are triggered at which caseload thresholds

Daily Patient Costs by Acuity

Model low, base, and high-acuity patient populations to understand how diagnosis mix affects financial performance

Monthly Admissions Required

Convert your target ADC into a specific monthly admission number based on your actual average length of stay

Marketer Productivity Targets

See exactly how many admissions per marketer are needed to sustain the census goal — with the math visible, not assumed

Cash Runway Projection

Model how long starting capital can sustain a branch operating below break-even while census builds — critical for new branches and turnarounds

Profitability Curve

Visualize how profit and operating margin change across the full census range from 10 to 200 patients

You have never had access to this information in a form you could use in the field. The simulator puts the complete financial picture of your branch in front of you — not as a report to review after the fact, but as a working model you can adjust, test, and use to understand the real stakes of your daily production.

WHAT THIS ALL MEANS

The Census Is Not the Point. The Care Is.

Everything in this document — the staffing model, the daily costs, the break-even math, the revenue rates, the target margin — exists to support a single reality: somewhere in your territory, right now, there is a person who qualifies for hospice care and is not receiving it. Their family is managing their medications without guidance. Their pain is being managed inadequately or not at all. They are facing the end of life without a team of professionals who know their name and their wishes.

That gap is not clinical. Physicians understand hospice. Nurses understand hospice. The gap is conversational. A referral that was not made because the rep did not have the right language. A physician who said 'not yet' because no one responded with clinical confidence. A patient who was never asked because the rep's calendar was full of low-value visits to accounts that were never going to convert.

The census is not the point. The census is what makes the point possible. The point is that a nurse shows up. A social worker calls. A chaplain sits. A family gets through the hardest days of their lives with people beside them who know exactly what they are doing and why they are there.

Your work in the field is the beginning of that chain. Every conversation you prepare for. Every referral you follow through on. Every physician you educate, every discharge planner you support, every family you help get to the right decision at the right time — that work funds the clinical team that carries it forward.

When you understand that, the number is not arbitrary. It never was. You just needed someone to explain it.

Key Terms Reference

Term	Definition
Average Daily Census (ADC)	The average number of patients in hospice service on any given day. The primary revenue driver for every hospice branch.
Break-Even ADC	Census at which revenue covers all costs — salaries, patient costs, and overhead. Below this, the branch loses money daily.
Target Margin ADC	The census needed to achieve the operating margin goal (typically 12-18%). This is the real sales target — not break-even.
Contribution Margin	Revenue per patient day minus direct costs. Approx. \$120-\$155 per patient per day. This is what covers salaries and overhead.
Reimbursement Rate	The Medicare rate paid for each day a patient is enrolled in hospice. Days 1-60 are reimbursed at a higher rate.

Monthly new admissions needed	Admissions needed to maintain target ADC, calculated from ADC goal and average length of stay.
How long runway	How many months starting capital can sustain a branch operating below break-even before funds are exhausted.

Access the Branch Profitability Simulator and full field education library at spartancoaching.com

Spartan Coaching | Practical coaching and consulting for hospice growth professionals

Every input is customizable. Every output is actionable. Start with your real numbers.